

AgentThink Pharma — Flagship Case Package

Version 1.0 | June 2026 | Confidential — Not for Distribution

PART 1 — Ranking of All 10 Cases

Each case is scored 1–10 across eight criteria. Scores are based on the completed council deliberations, retrospective outcome data, and the practical requirements of a live executive demo.

Scoring Criteria

#	Criterion	Description
1	Public recognizability	Is the drug known to pharma executives without explanation?
2	Availability of evidence	Can pre-decision evidence be assembled from public sources?
3	Governance failure visibility	Is the governance failure clear, specific, and non-controversial?
4	Financial impact	Is the financial consequence large enough to command attention?
5	Regulatory significance	Does the case involve FDA/EMA decisions that are publicly documented?
6	Ease of explanation	Can a non-scientist understand the case in under 3 minutes?
7	Council value demonstration	Does the council verdict add visible value beyond what was obvious?
8	Scrutiny resistance	Can the case withstand a hostile expert reviewer?

Scores

Case	Drug	C1	C2	C3	C4	C5	C6	C7	C8	Total
1	Torcetrapib	10	10	10	10	9	10	10	9	78
3	Muraglitazar	6	9	10	8	10	8	9	9	69
4	Sofosbuvir	9	10	7	10	10	9	8	9	72
5	Aducanumab	10	8	8	9	10	7	7	6	65
7	Evolocumab	7	9	6	9	8	8	7	8	62
9	Ticagrelor	7	8	7	8	8	7	7	8	60
10	Entrectinib	5	8	6	6	8	7	8	8	56
2	Semagacestat	5	8	8	7	7	7	8	7	57
6	Verubecestat	6	8	5	7	7	7	5	5	50
8	Eteplirsen	6	6	6	5	9	6	6	5	49

Ranking Summary

Rank	Drug	Score	Case Type	Council Verdict	Outcome
1	Torcetrapib	78/80	Major Failure	WAIT (0/8/2)	Phase III terminated; 82 deaths
2	Sofosbuvir	72/80	Major Success	GO (8/2/0)	\$12.4B revenue; HCV cure
3	Muraglitazar	69/80	Terminated	NO-GO (0/4/6)	Withdrawn pre-approval
4	Aducanumab	65/80	Controversial	WAIT (3/5/2)	Approved; commercial failure
5	Evolocumab	62/80	Success	GO (7/3/0)	Approved; \$1.4B revenue
6	Ticagrelor	60/80	Success	WAIT (3/6/1)	Approved; \$1.5B revenue
7	Entrectinib	56/80	Success	GO (7/3/0)	Approved; oncology standard
8	Semagacestat	57/80	Terminated	WAIT (1/5/4)	Phase III terminated
9	Verubecestat	50/80	Failure	GO (6/2/2)	Phase III futility — council wrong
10	Eteplirsen	49/80	Controversial	GO (7/1/2)	Approved; contested evidence

PART 2 — Flagship Recommendation

The flagship case is Torcetrapib.

No other case in the library scores above 72. Torcetrapib scores 78/80. The gap between first and second place (6 points) is larger than the gap between second and fifth place (10 points across four cases). This is not a close decision.

PART 3 — Why Torcetrapib Is the Strongest Case

The Argument in One Sentence

Pfizer had a published safety signal, 800M invested, a Lipitor patent cliff creating financial pressure, and a council that would have said WAIT—and they advanced anyway; 21B in market capitalisation.

Eight Reasons It Is the Strongest Case

- 1. Public recognizability (10/10).** Torcetrapib is one of the most cited pharmaceutical failures in the academic literature. Every head of clinical development at a top-20 pharma company knows this case. No explanation required. The name alone creates credibility.
- 2. Availability of evidence (10/10).** The pre-decision evidence is entirely public. The Phase II blood pressure signal was published in the *New England Journal of Medicine* in 2004 (Brousseau et al.). The CETP mechanism was published in 2000. The Phase II trial results were disclosed in Pfizer's 2004 annual report. The ILLUMINATE trial design was registered on ClinicalTrials.gov in 2004. Every document required to build the evidence manifest exists in the public domain with DOIs.
- 3. Governance failure visibility (10/10).** The governance failure is specific, documented, and non-controversial. The Phase II blood pressure signal (+2 mmHg systolic) was published. Pfizer's internal review acknowledged it. The decision to advance was made despite the signal. The ILLUMINATE Data Safety Monitoring Board terminated the trial in December 2006 citing excess mortality. The causal mechanism — off-target aldosterone stimulation — was confirmed by Forrest et al. in 2008. There is no ambiguity about what happened or why.
- 4. Financial impact (10/10).** 800M indirect write – off. 21B in market capitalisation lost in a single trading session on December 4, 2006. Pfizer's stock fell 10.6% in one day. These numbers are verifiable, public, and large enough to command the attention of any CFO, portfolio manager, or investment committee.

5. Regulatory significance ($\frac{9}{10}$). The case predates the FDA's enhanced cardiovascular safety requirements for lipid-modifying agents. The ILLUMINATE termination directly influenced the FDA's subsequent guidance on cardiovascular endpoint requirements for this drug class. The regulatory context is rich and documented.

6. Ease of explanation ($\frac{10}{10}$). The case can be explained to a non-scientist in three sentences: *"Pfizer was developing a drug to raise good cholesterol. In Phase II trials, the drug also raised blood pressure. Pfizer advanced to Phase III anyway. 82 people died. The company lost \$21 billion in one day."* No scientific background required.

7. Council value demonstration ($\frac{10}{10}$). The council verdict (WAIT, 0/8/2) is retrospectively correct. The primary blocker — the unexplained blood pressure signal — is exactly the signal that caused the Phase III termination. The council identified the right risk, for the right reason, using only pre-decision evidence. This is the clearest possible demonstration of council value.

8. Scrutiny resistance ($\frac{9}{10}$). The one vulnerability is the LLM contamination risk: the model may have been trained on post-failure data. This must be disclosed. However, the evidence boundary is the most thoroughly documented of any case in the library (23 admitted documents, all with DOIs, all pre-December 2005). The case can withstand expert review better than any other case in the library, provided the LLM limitation is disclosed upfront.

The One Weakness

The LLM contamination risk is real and must be disclosed in every presentation. The exact language: *"The council was run using a large language model. We cannot verify that the model's training data did not include post-failure information about torcetrapib. We have documented the evidence provided to the council. The evidence boundary is available for independent review."*

This disclosure does not destroy the case. It demonstrates intellectual honesty, which is itself a credibility signal.

PART 4 — 30-Minute Executive Demo Built Around Torcetrapib

Audience

Chief Medical Officer, Head of Clinical Development, or Head of R&D Strategy at a top-20 pharma company or AI-native biotech. One to three people in the room. The presenter is the AgenThink CEO.

Pre-Meeting Preparation

Send the following 24 hours before the meeting:

- Torcetrapib case summary (1 page, PDF)
- Meeting agenda (3 lines: context, demo, pilot discussion)
- No other materials

Minute-by-Minute Script

MINUTES 0:00–2:00 — OPENING: THE QUESTION

[Do not open with a company introduction. Open with the case.]

"Before we start, I want to ask you one question. In 2005, Pfizer had Phase II data showing their CETP inhibitor raised HDL cholesterol by 72%. It also raised systolic blood pressure by 2 millimetres of mercury. They had \$800 million invested and a Lipitor patent cliff creating enormous financial pressure. Would you have advanced to Phase III?"

[Pause. Let them answer. Do not fill the silence.]

"That's the question we built AgenThink to answer. Not with a recommendation — with a governance record."

MINUTES 2:00–5:00 — THE PROBLEM: WHAT GOVERNANCE LOOKS LIKE TODAY

[Show Slide 1: The Decision Gap]

"Every major pharmaceutical company has a governance process for Phase II to Phase III advancement decisions. It typically involves a committee, a review meeting, and a go/no-go vote. What it almost never produces is a documented record of: which

specific risks were raised, which constitutional rules were applied, which personas were overruled, and why the decision was made despite the objections.”

“Torcetrapib had a governance process. Pfizer had a review committee. The blood pressure signal was discussed. The decision was made to advance. And there is no public record of which specific risk assessment framework was applied to the blood pressure signal, or why it was deemed acceptable.”

“That is the governance gap. And it is not unique to Pfizer.”

MINUTES 5:00–8:00 — THE EVIDENCE MANIFEST: WHAT WE GAVE THE COUNCIL

[Show Slide 2: Evidence Manifest — 23 documents, all pre-December 2005]

“Before we ran the council, we built an evidence manifest. Every document we gave the council is listed here, with a source URL, a publication date, and a verification status. Nothing published after December 31, 2005 was admitted. The ILLUMINATE trial results — December 2006 — are not in this manifest.”

“I want to be transparent about one limitation. The council was run using a large language model. We cannot verify that the model's training data did not include post-failure information. We have disclosed this in writing. The evidence manifest is available for independent review.”

[Show the manifest — 23 rows, DOIs visible, cutoff date visible]

MINUTES 8:00–14:00 — THE COUNCIL: 10 PERSONAS, ONE DECISION

[Show Slide 3: Council Chamber — 10 persona cards]

“The council is 10 specialist personas. Each has a defined role, a defined voting weight, and a defined set of constitutional rules they are required to apply. They are not a committee. They are a structured deliberation engine.”

[Walk through 3 persona cards: Drug Safety Expert, Chief Biostatistician, Portfolio Manager]

Drug Safety Expert (WAIT, 95% confidence):

“The +2 mmHg blood pressure elevation observed in Phase II is mechanistically unexplained. CETP inhibition should not directly affect blood pressure. This signal requires investigation before Phase III advancement. Constitutional rule PC-002 applies: unexplained safety signals require mechanistic investigation before advancement.”

Chief Biostatistician (WAIT, 95% confidence):

“The blood pressure signal is statistically significant and dose-dependent. The Phase II dataset is insufficient to characterise the full magnitude of the effect in a Phase III population. The confidence interval for the blood pressure effect includes clinically significant values. Constitutional rule PC-001 applies: statistically significant safety signals require pre-specified stopping rules.”

Portfolio Manager (WAIT, 90% confidence):

“The financial pressure from the Lipitor patent cliff creates a documented bias risk. The investment of \$800M creates a sunk cost fallacy risk. Constitutional rule PC-003 applies: financial pressure must be documented and disclosed as a potential bias in the advancement decision record.”

[Show vote distribution: 0 GO / 8 WAIT / 2 NO-GO]

“The verdict was WAIT. Not NO-GO. The council did not say the drug would fail. It said: investigate the blood pressure signal before advancing. That is a different recommendation.”

MINUTES 14:00–17:00 — THE GOVERNANCE RECORD: THREE CONSTITUTIONAL RULES

[Show Slide 4: Constitution Audit — 3 rules triggered]

“The council identified three constitutional blockers. Rule PC-001: statistically significant safety signals require pre-specified stopping rules. Rule PC-002: unexplained safety signals require mechanistic investigation. Rule PC-003: financial pressure must be documented as a potential bias.”

“These are not opinions. They are governance rules. The Institutional Proof Report records which rules were triggered, which personas invoked them, and what evidence was cited. This is the audit trail.”

MINUTES 17:00–20:00 — THE OUTCOME: WHAT HAPPENED

[Show Slide 5: Outcome Comparison — council verdict vs. actual outcome]

“The ILLUMINATE trial was terminated on December 2, 2006. The Data Safety Monitoring Board cited excess mortality: 82 deaths in the torcetrapib arm versus 51 in the control arm. The blood pressure effect in Phase III was +5.4 mmHg — not 2. Pfizer wrote off 800million.The stock fell 10.621 billion.”

“The council said WAIT. The council was right. The primary blocker — the unexplained blood pressure signal — was the mechanism that caused the Phase III termination. The off-target aldosterone mechanism was confirmed by Forrest et al. in 2008.”

[Pause.]

“We are not claiming the council would have saved those 82 lives. We are claiming the council would have produced a governance record that gave Pfizer’s leadership a documented reason to pause.”

MINUTES 20:00–23:00 — THE PILOT PROPOSAL

[Show Slide 6: Pilot Proposal — one case, 30 days, \$25,000]

“We are not asking you to deploy this system. We are asking you to run one case.”

“Pick a drug from your pipeline — or from your history. A Phase II to Phase III decision you made in the last five years. We will run the council against the pre-decision evidence. We will produce an Institutional Proof Report. You will evaluate whether the council identified the risks your governance process identified, and whether it identified any risks your process missed.”

“The pilot is 30 days. The cost is \$25,000. The deliverable is one Institutional Proof Report and a methodology review session.”

MINUTES 23:00–30:00 — Q&A

[Stop presenting. Ask one question.]

“Is there a case from your pipeline — or your history — that you would want us to run?”

[Listen. Do not fill the silence. The answer to this question determines whether the meeting was successful.]

Demo Success Criteria

Outcome	Probability	Next Step
Prospect names a case	40–55%	Send pilot proposal within 24 hours
Prospect asks for methodology paper	25–35%	Send GitHub link + methodology PDF within 24 hours
Prospect asks for more case examples	20–30%	Send Muraglitazar and Sofosbuvir case summaries within 24 hours
No engagement	5–10%	Diagnose: wrong audience, wrong case, wrong timing

Required Demo Materials

#	Asset	Status	File
1	Torcetrapib case summary (1 page)	Done	torcetrapib-case-study-summary.pdf
2	Evidence manifest (23 documents)	Done	In IPR PDF
3	Council vote distribution slide	To create	From IPR data
4	Constitution audit slide	To create	From IPR data
5	Outcome comparison slide	To create	From IPR data
6	Pilot proposal (1 page)	To create	PAT-001 summary
7	LLM limitation disclosure	To create	LDS-001

PART 5 — One-Page Visual Summary

This section specifies the content and layout of the one-page visual summary. It is designed for print (A4) and digital (PDF) distribution.

Layout Specification

Title (top, full width):

AgenThink Council — Torcetrapib Retrospective

Subtitle:

Phase II → III Advancement Decision | Evidence Cutoff: December 31, 2005 | Council Run: June 2026

Left column (40% width):

THE DECISION

Drug: Torcetrapib (Pfizer) Mechanism: CETP inhibitor Goal: Raise HDL-C, reduce CV events Phase II result: HDL +72%, BP +2 mmHg Decision date: 2005 Decision: Advance to Phase III (ILLUMINATE)

THE SIGNAL

Blood pressure elevation: +2 mmHg systolic (Phase II) Mechanistic explanation: None available pre-decision Constitutional rule triggered: PC-002 (unexplained safety signal)

Centre column (20% width) — VOTE DISPLAY:

Large circular graphic:



VERDICT: WAIT Proof Score: 93/100

Right column (40% width):

COUNCIL BLOCKERS

- 1. PC-001 — BP signal requires pre-specified stopping rules
- 2. PC-002 — Unexplained safety signal requires mechanistic investigation
- 3. PC-003 — Financial pressure documented as potential bias

THE OUTCOME

ILLUMINATE terminated: December 2, 2006 Deaths: 82 (torcetrapib) vs. 51 (control) Write-off: 800MMarketcaploss : 21B
Mechanism confirmed: Forrest et al. 2008 (off-target aldosterone)

Footer (full width):

Evidence boundary: 23 documents, all pre-December 2005, all DOI-cited. LLM limitation disclosed. Full methodology:
github.com/agentthinkmesh/pharma-council-v1

Bottom right:

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PART 6 — Institutional Proof Report Redesign for Torcetrapib

The existing IPR (4 pages, generated June 2026) is functional but dense. This section specifies the redesigned version optimised for executive presentation.

Redesign Principles

The existing IPR was generated for internal validation. The redesigned IPR is for external presentation to pharma executives. The differences are:

Dimension	Existing IPR	Redesigned IPR
Length	4 pages	6 pages
Audience	Internal validation	External pharma executive
Opening	Technical summary	One-sentence verdict
Evidence manifest	Inline table	Dedicated page
Vote display	Text table	Visual vote card
Limitations	Section 14 (buried)	Page 1 (prominent)
Retrospective appendix	Section 15	Dedicated final page
Branding	None	AgenThink header/footer

Redesigned Structure (6 pages)

Page 1 — Executive Summary

- Case name, drug, company, decision date
- Evidence cutoff date (prominent, boxed)
- LLM limitation disclosure (prominent, boxed)
- Council verdict (large, bold: WAIT)
- Proof Score: 93/100
- Three-line summary: what the council found, what the council recommended, what happened

Page 2 — Evidence Manifest

- Table: 23 admitted documents with source, date, DOI, verification status
- Excluded documents section (post-cutoff)
- Evidence boundary statement

Page 3 — Council Execution

- Visual vote display (0 GO / 8 WAIT / 2 NO-GO)
- 10 persona cards with vote, confidence, primary flag
- Voting weight table

Page 4 — Constitutional Audit

- Three triggered rules (PC-001, PC-002, PC-003) with full text
- Persona-rule mapping table
- Governance finding summary

Page 5 — Institutional Proof Report

- Decision drivers (3)
- Key blockers (3)
- Confidence assessment
- Release gate: WAIT pending mechanistic investigation of BP signal
- Audit references

Page 6 — Retrospective Outcome Appendix

- Clearly labelled: "POST-DECISION DATA — NOT USED IN COUNCIL INPUT"
- ILLUMINATE termination date and reason
- Death count (82 vs. 51)
- Financial impact (800M+21B)
- Mechanism confirmation (Forrest et al. 2008)
- Retrospective validation statement

PART 7 — How the Case Should Appear on the Website

Page: /pharma (or /validation)

Hero section:

Headline: "The council said WAIT."
 Subheadline: "Torcetrapib, 2005. Pfizer advanced anyway."
 Body: "82 deaths. \$800M write-off. \$21B market cap loss.
 AgenThink's council identified the governance failure
 using only pre-decision evidence."
 CTA: "See the full case →"

Case card (on homepage or product page):

[Case Study]
Torcetrapib — Pfizer — 2005

Council verdict: WAIT (0/8/2)
Primary blocker: Unexplained blood pressure signal
Outcome: Phase III terminated. 82 deaths. \$800M write-off.
Retrospective alignment: ✓ Correct

[Read the case →]

Dedicated case page (/pharma/torcetrapib):

Section 1 — The Decision (2 paragraphs, non-technical) Section 2 — The Evidence Manifest (table, 23 documents) Section 3 — The Council Vote (visual) Section 4 — The Constitutional Blockers (3 rules) Section 5 — The Outcome (retrospective data, clearly labelled) Section 6 — The Limitation (LLM disclosure, prominent) Section 7 — CTA: “Run a case from your pipeline →”

SEO metadata:

- Title: “Torcetrapib Governance Failure — AgenThink Retrospective Analysis”
- Description: “AgenThink’s council identified the Phase II blood pressure signal that caused the ILLUMINATE trial termination, using only pre-decision evidence from 2005.”
- Keywords: torcetrapib, CETP inhibitor, pharmaceutical governance, Phase III failure, clinical development

What NOT to put on the website:

- Claims about accuracy rates (n=10 is insufficient)
- Claims about prospective use
- Claims about regulatory acceptance
- The verubecestat failure (do not hide it, but do not feature it on the homepage)

PART 8 — How the Case Should Appear in Outreach Emails

Email 1 — Biotech / AI-Native Pharma

Subject: Torcetrapib retrospective — what a structured council would have said in 2005

Body:

Dear [First Name],

In 2005, Pfizer had Phase II data showing torcetrapib raised HDL cholesterol by 72%. It also raised systolic blood pressure by 2 mmHg. Pfizer advanced to Phase III.

The ILLUMINATE trial was terminated in December 2006. 82 deaths in the torcetrapib arm versus 51 in the control. 800M write-off. 21B market cap loss in one trading session.

We ran AgenThink’s council against the pre-decision evidence — everything available before December 31, 2005. The verdict was WAIT (0 GO / 8 WAIT / 2 NO-GO). The primary blocker: an unexplained blood pressure signal requiring mechanistic investigation before Phase III advancement.

The council was right. For the wrong reason, or the right reason — that is what the pilot is designed to test.

We have completed 10 retrospective cases. The methodology is published on GitHub. The evidence boundary is documented.

If you have a Phase II → III decision from your pipeline — historical or current — I would like to run it.

30 minutes to show you the case. No commitment required.

[Calendar link]

Best, Farouq Sultan CEO, AgenThink

Attachment: Torcetrapib case summary (1 page)

Email 1 — CRO (ICON / Medpace)

Subject: Governance layer for Phase II → III transitions — torcetrapib case

Body:

Dear [First Name],

The torcetrapib failure cost Pfizer 800M and 21B in market cap. The Phase II blood pressure signal was published. The governance process did not produce a documented record of why the signal was deemed acceptable.

AgenThink has built a structured multi-persona council that produces a governance record for Phase II → III advancement decisions. We have completed 10 retrospective cases, including torcetrapib. The council verdict was WAIT. The outcome was Phase III termination.

We are exploring partnership models with CROs who want to offer a governance layer alongside their clinical services. The model is straightforward: you provide the clinical execution, AgenThink provides the governance record.

I would like 30 minutes to show you the torcetrapib case and discuss whether there is a partnership structure that makes sense for [Company].

[Calendar link]

Best, Farouq Sultan CEO, AgenThink

Attachments: Torcetrapib case summary (1 page), Validation library summary (2 pages)

Email 1 — Large Pharma (after first pilot complete)

Subject: Independent governance council for Phase II → III decisions — retrospective validation

Body:

Dear [First Name],

We have completed a retrospective pilot of AgenThink's governance council on the torcetrapib Phase II → III advancement decision. The council verdict was WAIT. The primary blocker was the unexplained blood pressure signal. The ILLUMINATE trial was terminated in December 2006 for excess mortality.

The council was right. We have documented the evidence boundary, the constitutional rules applied, and the full deliberation record. The methodology is published on GitHub.

We have now completed 10 retrospective cases across oncology, cardiovascular, and neurology. The validation library is available for review.

I would like 30 minutes to show you the case and discuss whether a retrospective pilot on a [Company] drug would be of interest.

[Calendar link]

Best, Farouq Sultan CEO, AgenThink

Attachments: Torcetrapib Institutional Proof Report (4 pages), Validation library summary (2 pages), Pilot proposal (1 page)

LinkedIn Message (connection request note)

"Hi [First Name] — I've been working on a governance council for Phase II → III advancement decisions. We ran a retrospective on torcetrapib — council said WAIT in 2005, using only pre-decision evidence. Wanted to share the case. Happy to connect."

LinkedIn Follow-Up (after connection accepted)

"Thanks for connecting. The torcetrapib case summary is attached — 1 page. The council verdict was WAIT (0/8/2). The primary blocker was the blood pressure signal. The ILLUMINATE trial was terminated 13 months later. If you have 30 minutes, I'd like to show you the full deliberation record."

Appendix — Flagship Case Selection Rationale

The decision to select torcetrapib over sofosbuvir (the second-ranked case) is based on the following logic:

Sofosbuvir is a success case. The council issued a clean GO. The drug was approved and generated \$12.4B in revenue. This is an important case for demonstrating that the council does not produce false-positive blocking. However, it is not the right flagship case.

The flagship case must create urgency. Urgency comes from failure, not success. A pharma executive who sees the torcetrapib case understands immediately what the governance gap costs. A pharma executive who sees the sofosbuvir case thinks: *"We already have a process for identifying good drugs."*

The torcetrapib case answers the question that matters: “*What would this council have done with a drug that killed people?*” The answer is: it would have said WAIT. That is the case that opens doors.

Sofosbuvir should be the second case shown — immediately after torcetrapib — to demonstrate that the council does not block good drugs. The two cases together tell the complete story: the council stops failures and approves successes.

AgenThink Pharma Flagship Case Package v1.0 | June 2026 Confidential — Not for Distribution All council deliberations use pre-decision evidence only. LLM limitation disclosed. Full methodology: github.com/agentthinkmesh/pharma-council-v1